PIR - Performance Indicator Record

Review identification, purpose and boundary

PIR · v1.0

Study / protocol	CIP version	Site / code	PIR ID
Period reviewed / data cut-off	**Previous PIR**	**Rule set (version, effective date)**	**Review date**
Review type ☐ Routine ☐ Early cases ☐ Triggered ☐ Action check	**Prepared by / role**	**Accountable reviewer / role**	**Controlled location**

A Purpose and use

The PIR reviews the site's performance over one period through six indicators fixed before the first participant, plus any the site adds. It takes the results already reported by the source records (FUCR, SLCT, CSST, IMC, SART and others) and does not recount them. What it adds is the reading across time and across indicators: how each result moves over the last periods (D), and whether problems in different indicators share the same participants, surgeon, staff, time or process step (E). From that it decides what to correct (F), checks whether earlier actions worked (G) and concludes on the period (H).

Every result keeps its count and denominator; a percentage alone hides small numbers. A rule is never changed to fit a result; a new rule applies from its effective date.

Unit of analysis	Decision supported	Inputs	Outputs
One site, one review period, each indicator with its own denominator.	Whether each indicator is within its rule, needs closer review or action, and what the site must improve.	Results of the source records; the previous PIR; the rule set.	Trend and links (D, E); improvement actions with a check (F, G); routes to CAPA, SRR, POT, SLCT or the sponsor.

B Review boundary

Fix the boundary before reading any result.

Records included (IDs or ranges, cut-off)	
Records missing, late or excluded, and the effect on denominators	
Rule or source changes in the period	
Periods compared and whether they are comparable	

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Indicator rules

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C Indicator rules

Set before the first participant and approved by the PI. Write each condition so that two reviewers reach the same decision. Rows 7 and 8 are for indicators the site adds.

Indicator	Source records	Unit (denominator)	Within rule when	Closer review when	Action required when	Rule ID · version
1 · Enrollment quality	PSEAR, SIFSL, deviation record					
2 · Protocol deviations	Deviation record, ICDL, IMC					
3 · Follow-up compliance	FUCR, PFVR					
4 · Safety reporting timeliness	SART					
5 · Surgeon procedural consistency	SLCT, ICDL, CSST					
6 · Critical data completeness	IMC, query log, operative notes					

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Results across periods

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D Results across periods

Copy each result from its source record as count / denominator (%). Direction compares the last four periods: a change counts when it passes the closer-review margin or lasts two periods in a row; with fewer than three periods write Too few periods.

Indicator	Three periods back	Two periods back	Last period	This period	Cumulative	Direction	Decision (C)
1 · Enrollment quality						☐ Better ☐ Stable ☐ Worse ☐ Too few periods	☐ Within rule ☐ Closer review ☐ Action required ☐ Not assessable
2 · Protocol deviations						☐ Better ☐ Stable ☐ Worse ☐ Too few periods	☐ Within rule ☐ Closer review ☐ Action required ☐ Not assessable
3 · Follow-up compliance						☐ Better ☐ Stable ☐ Worse ☐ Too few periods	☐ Within rule ☐ Closer review ☐ Action required ☐ Not assessable
4 · Safety reporting timeliness						☐ Better ☐ Stable ☐ Worse ☐ Too few periods	☐ Within rule ☐ Closer review ☐ Action required ☐ Not assessable
5 · Surgeon procedural consistency						☐ Better ☐ Stable ☐ Worse ☐ Too few periods	☐ Within rule ☐ Closer review ☐ Action required ☐ Not assessable
6 · Critical data completeness						☐ Better ☐ Stable ☐ Worse ☐ Too few periods	☐ Within rule ☐ Closer review ☐ Action required ☐ Not assessable
						☐ Better ☐ Stable ☐ Worse ☐ Too few periods	☐ Within rule ☐ Closer review ☐ Action required ☐ Not assessable
						☐ Better ☐ Stable ☐ Worse ☐ Too few periods	☐ Within rule ☐ Closer review ☐ Action required ☐ Not assessable

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Links between indicators

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E Links between indicators

For every indicator not within its rule, look for the same participants, surgeon, staff member, time point, component or process step in the other indicators and in the source records. Write what was compared and the counts. A shared pattern is a lead to test, not a proof of cause.

#	Pattern seen	Indicators involved	What was compared (records and counts)	Likely shared cause	Conclusion
1					☐ Supported ☐ Not supported ☐ Open: needs more data
2					☐ Supported ☐ Not supported ☐ Open: needs more data
3					☐ Supported ☐ Not supported ☐ Open: needs more data
4					☐ Supported ☐ Not supported ☐ Open: needs more data

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Improvement actions and their effect

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F Improvement actions

One action per cause found in D or E. Set the check that will show it worked before starting it. The receiving record owns the action.

Action	What will change (from D or E)	Receiving record	Owner	Due	How we will know it worked (indicator, value, period)

G Did earlier actions work?

Compare the indicator before and after with the check set in F. Done is not the same as effective.

Action	Check set when assigned	Value before	Value now	Result	Next step
				☐ Worked ☐ Did not work ☐ Too early	
				☐ Worked ☐ Did not work ☐ Too early	
				☐ Worked ☐ Did not work ☐ Too early	
				☐ Worked ☐ Did not work ☐ Too early	

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Period conclusion

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H Period conclusion

The conclusion covers only the stated period, records and indicators; it does not certify the site.

Decision	Count	Indicators
Within rule		
Closer review		
Action required		
Not assessable		

Site performance over the periods	☐ Improving ☐ Stable ☐ Declining ☐ Mixed
What the site learned this period	
Routes opened (CAPA, SRR, POT, SLCT, sponsor) or none	
Next review (date or trigger)	

Prepared by

Name / role:

Signature / date:

Principal Investigator

Name:

Signature / date: